A randomised comparison of two dosing schedules of astelvia in post-operative prophylaxis

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Annals of Fictional Clinical Medicine 2026; doi:10.9999/afcm.2026.0164

Abstract

In this randomised, open-label trial, 412 participants undergoing elective surgery received astelvia 1 g either three times daily or twice daily. The primary endpoint was surgical site infection at 30 days. Adverse events were collected as aggregate counts. Rash occurred in 18 of 206 participants in the three-times-daily arm and 11 of 206 in the twice-daily arm. No individual patient is identifiable from the data presented.

Keywords: astelvia, randomised trial, prophylaxis, aggregate data

Introduction

Optimal dosing frequency for post-operative prophylaxis remains uncertain. This trial compared two schedules already in common use. The protocol was registered before recruitment began.

Discussion

The absolute difference in the primary endpoint was small and the confidence interval crossed the null. The adverse event counts are reported as aggregate frequencies only; no individual case narratives were collected, and these counts should not be treated as individual case safety reports.

Conclusion

Neither schedule was clearly superior. Aggregate safety data from this trial do not constitute reportable individual cases.

Acknowledgements

The authors thank the clinical pharmacy team at the participating centre for their assistance with record retrieval.

Funding

This work received no specific grant from any funding agency in the public, commercial or not-for-profit sectors.

Conflict of interest

The authors declare that they have no competing interests.

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